

BEYOND THE BREAST: EXTENSIVE BREAST FILLER MIGRATION WITH SECONDARY INFECTION

Anusha Apparau¹, Wan Mezlina binti Wan Zakaria², Noor Laily Syakimah binti Saidi², Zahurin binti Ismail²

¹Department of Radiology, Hospital Sungai Buloh, Malaysia ²Department of Radiology, Hospital Putrajaya, Malaysia



INTRODUCTION

Breast filler injections continue to be performed in Malaysia despite recognised safety concerns. Delayed filler migration with secondary infection is uncommon and poses significant diagnostic challenges. We report the complementary roles of ultrasound and MRI in delineating disease extent and guiding multidisciplinary management.

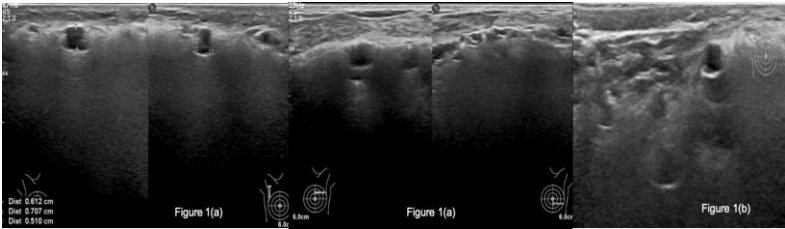
CASE PRESENTATION

A 25-year-old woman presented with a four-month history of progressive right breast pain and swelling following bilateral cosmetic breast filler injections at a private aesthetic clinic. Initial aspiration and oral antibiotics were unsuccessful, with subsequent extension of symptoms to the right axilla and upper arm.

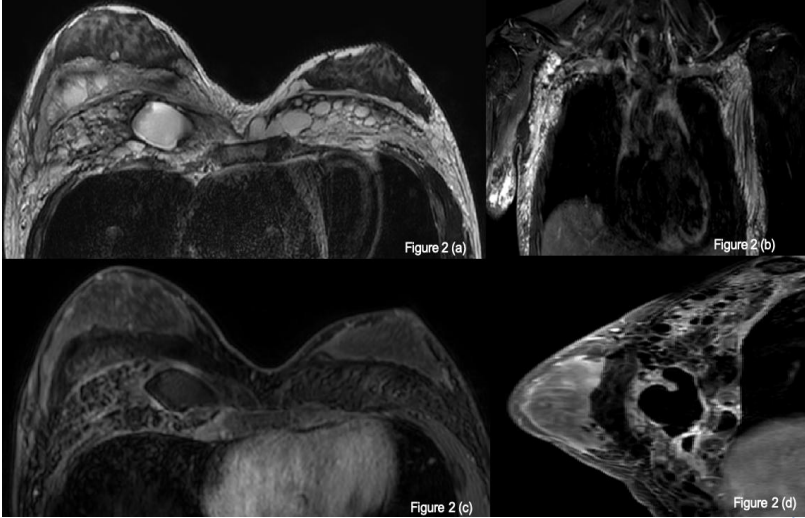
FINDINGS AND PROGRESS

Breast ultrasound demonstrated bilateral snowstorm appearances and contrast-enhanced breast MRI demonstrated extensive filler migration involving the retromammary spaces, chest wall, bilateral axillae, supraclavicular and infraclavicular regions, and the right upper arm extending to the elbow with features superimposed infection. The patient received intravenous antibiotics followed by incision and drainage of an infected migrated filler deposit. Follow-up ultrasound demonstrated reduced filler burden with improving cellulitis and a residual postoperative seroma.

FINDINGS



Bilateral snowstorm appearances (Fig 1a) with filler migration to right axilla (Fig 1b)



Contrast-enhanced breast MRI. Figures 2a and 2b depicting extensive filler migration involving the retromammary spaces, chest wall, bilateral axillae, supraclavicular and infraclavicular regions, and the right upper arm extending to the elbow. Figures 2c and 2d demonstrating peripheeral enhancement of the right-sided filler deposits with reactive axillary lymphadenopathy indicated superimposed infection.

DISCUSSION AND CONCLUSION

Breast filler migration is an uncommon delayed complication that may mimic breast malignancy or other soft tissue pathology. Ultrasound identifies the characteristic snowstorm appearance and facilitates image-guided intervention, while MRI delineates disease extent, characterises superimposed infection and guides multidisciplinary management. Recognition of these imaging features helps differentiate filler-related complications from breast malignancy, avoiding unnecessary interventions.

Ultrasound and MRI enable accurate diagnosis, assessment of disease extent and timely multidisciplinary management of this rare but clinically significant complication.

REFERENCES

- 1.KKhorasanizadeh F, et al. Clin Imaging. 2026;131:110708.
2. Distant migration of gel filler: imaging findings following breast augmentation. 2022.
- 3.Momeni A, et al. Insights Imaging. 2017;8:557–568.
4. Complications after breast augmentation with dermal fillers containing copolyamide: a systematic review. 2024.
5. Extensive migration of injected free liquid silicone for breast augmentation with related major complications. 2018.

ACKNOWLEDGEMENTS

I would like to express my sincere appreciation to my supervisors (Drs Zahurin binti Ismail, Wan Mezlina binti Wan Zakaria and Noor Laily Syakimah binti Saidi) for their guidance and support and to the Department of Radiology, Hospital Putrajaya for their valuable assistance in the evaluation of this case.

QUICK POSTER CHECK

☐ Minimal text ☐ Results dominate ☐ Clear figures/tables ☐ One-sentence conclusion ☐ Readable fonts ☐ QR/contact optional